London School of Hygiene & Tropical Medicine

Cover sheet for work submitted towards Assessment 2025-26

LSHTM Candidate Number:	493467
Module Title:	Control of Sexually Transmitted Infections (STIs)
Module Number:	3192
Word count *(when a word limit is specified)* Please give the number of words you have used in your assessment	2926

Please attach a copy of this sheet to *each* piece of work submitted.

Submitting your assessment is regarded by your programme as confirmation that you have read, understood and agreed with the general LSHTM Academic Misconduct Policy and the LSHTM guidance on using AI tools responsibly to support your learning.

AI tools should only be used in ways that uphold academic integrity. All submitted work must be your own. Submitting copied or unacknowledged material — including AI-generated content — constitutes academic misconduct under **LSHTM Academic Manual** Chapter 7: 7.2.4.5. If AI use means you are no longer the true author of your work, this will be treated as misconduct. If unsure about permitted AI use, consult the LSHTM guidance on using AI tools responsibly or speak with your module organiser or programme director.

By submitting work for assessment you are confirming that:

- You understand and comply with LSHTM's definitions of plagiarism and cheating.
- All work submitted represents your genuine knowledge and skills, and where the work of others has been included, it is referenced appropriately
- You give consent for the School to upload relevant documents and information to external services or third parties, in order to identify potential plagiarism or irregularities.
- You have acknowledged any use of Generative AI tools appropriately.

Failure to comply with the School's academic policies and regulations may be penalised.

Penalties for exceeding the word count apply to both module assessments and projects. Please check stated word counts in your assessment instructions. See Chapter 8a of the Academic Manual for details.

You should consult your tutor or Module Organiser if you are in any doubt about what is permissible.

PLEASE START YOUR ASSESSMENT ON THE NEXT PAGE

Alianza por la Salud y Dignidad de las Trabajadoras Sexuales (ASD-TS)

GATED

Control of STI Programme Proposal

A 3-year targeted STI prevention, screening and linkage-to-care programme for venue based and non-registered/clandestine female sex workers in Lima Metropolitan Area

493467

Table of Contents

1. Executive Summary 3

2. Background and Situational Analysis 4

3. Rationale, Goals and Objectives 5

4. Intervention Plan 6

5. Monitoring and Evaluation 7

6. Budget Justification 8

7. References 9

8. Appendices 10

Executive Summary

This proposal is submitted by *Alianza por la Salud y Dignidad de las Trabajadoras Sexuales (ASD-TS)*. We are a non-governmental organisation that focus on advocating for accessible STI services for sex workers across South America. We have years of expertise in conducting STI screening, treatment and prevention programmes. We are committed to delivering community-based interventions and have extensive experience working in partnership with government and other non-governmental organisations (NGOs) to deliver sustainable sexual health interventions.

We are seeking £10 million in funding over a 3-year period to implement a community-based STI control programme targeting female sex workers (FSWs) in Lima, Peru. FSWs experience a high burden of STIs and they are considered an important bridging population in transmitting STIs to the general population[1]. A large number of FSWs in Lima operate in the informal sector and face significant barriers in accessing existing healthcare services due to lack of legal registration and stigma.

The proposed intervention will deliver integrated sexual health services through two mobile clinics and peer-led outreach. Services will include regular STI screening and treatment and provision of pre-exposure prophylaxis (PrEP) and cervical screening. Commodities such as condoms and lubricants will also be distributed. The programme will also establish referral pathways for advanced care including cervical cancer management, mental health services and ART services.

By decentralising service delivery and working in partnership with community organisations such as Miluska Vida y Dignidad (MVD) and the Ministry of Health of Peru (MINSA) the programme aims to reduce the prevalence of curable STIs such as chlamydia, gonorrhoea, trichomoniasis and syphilis by 30% over 3 years. It also aims to improve treatment outcomes for FSWs, expand access to integrated services and improve sexual health outcomes for years to come.

INTERVENTION TITLE	A 3-year targeted STI prevention, screening and linkage-to-care programme for venue based and non-registered/clandestine female sex workers in Lima Metropolitan Area
COUNTRY WHERE PROGRAMME IS TO BE IMPLEMENTED	Peru
DURATION OF GRANT REQUEST (*in months*)	36
TOTAL PROGRAMME BUDGET *(in GBP sterling)*	10,000,000

Background and Situational Analysis

Sexually transmitted infections (STIs) are a considerable public health concern in Peru. Female sex workers (FSWs) bear a considerable burden of STIs, including syphilis, chlamydia, trichomoniasis and gonorrhoea, along with human immunodeficiency virus (HIV) and human papillomavirus (HPV)[2]. In the capital city of Lima, there are estimated to be between 20,000-30,000 women who engage in sex work both in formal (regulated) and informal (unregulated) settings, however, precise figures are difficult to ascertain due to the hidden and changing nature of informal work[2]. This surge in STIs when compared to the general population is driven by structural vulnerabilities such as economic insecurity, high-client turnover, high exposure risk and barriers to accessing healthcare[2,3]. This allows for consistent cycles of infection and reinfection within this population. In order to reduce the prevalence and incidence of disease burden faced these interconnected factors must be addressed in order to provide safe and efficient STI control[4]. In Peru, multicity surveillance data reported that chlamydia prevalence was 16.4% amongst FSWs compared with 6.5% in the general population, with gonorrhoea prevalence at 1.6% versus 0.1%[5]. Trichomoniasis, which is often misdiagnosed due to limited routine screening, is estimated at 8% [5]. HIV prevalence amongst FSWs is approximately 0.7% compared to 0.2% amongst women in the general population and syphilis is a pressing concern with a prevalence of 7.3%[4,6]. Additionally, HPV prevalence amongst FSWs is substantially higher than in the general population at 40-50% which leads to an increased risk of cervical cancer [9]. Cervical cancer is the leading cause of mortality for women of reproductive age, therefore highlighting the need for integrated screening into sexual health services [6,10]. The epidemiological data collected here is limited due to underreporting which suggests that the true burden of STIs is underestimated. Sex work is legal in Peru for people over the age of 18 if they work in a licensed establishment, register with municipal authorities and undergo periodic screening provided by the Ministry of Health (MINSA)[7,8]. However, the vast majority of FSWs operate in the informal sector which includes bars, streets and clubs. Informal FSWs when compared with registered FSWs have less consistent access to healthcare services. General difficulty accessing services, fear of discrimination, limited trust in healthcare systems, missing documentation, and opportunity costs involved with attending health services represent major barriers to care for this community. Despite sex work being legal, FSWs face considerable marginalisation and social stigma which the system fails to protect them from. This can promote exploitation in brothels, clubs and other venues leading to an increase in risk behaviour and rejection of STI screening [11,12]. While testing coverage for formal FSWs was reported at 100% it was not possible to ascertain a precise figure for informal FSWs [14]. Existing responses include STI/HIV services delivered through public health facilities and targeted programmes, as well as outreach programmes delivered by NGOs [14]. The Ministry of Health is the primary provider of screening services in Peru and operate a national STI/HIV programme however this programme is not accessible to everyone due to registration status, and fear of discrimination. Similarly, EsSalud is a social health insurance system that covers individuals employed in the formal sector, this excludes the majority of FSWs from accessing healthcare as many of them work in the informal sector and are therefore not eligible [14]. Furthermore, these services are highly fragmented with HIV testing, cervical screening, HPV vaccination and STI testing delivered separately. Despite ongoing efforts, STI transmission within this population remains consistent, implying that without targeted interventions the burden is unlikely to decrease.

Rationale, Goals and Objectives

Despite existing sexual health services provided by MINSA and local NGOs, gaps remain in reaching FSWs, especially those of whom operate in the informal sector [14]. Regular STI screening is suboptimal, leading to ongoing transmission and untreated infections, whilst, current service delivery is fragmented and facility-based which limits access. There is strong evidence that peer led, community-based interventions

are useful in improving uptake of sexual health services amongst FSWs. By combining this with a mobile delivery model it addresses both demand and supply barriers [14]. Integration of STI screening, testing and prevention services with PrEP treatment and cervical screening into a singular platform increases efficiency immensely and makes the service far more accessible, therefore increasing uptake. This approach would fit the needs of FSWs in Lima as a large proportion remain underserved by existing services. An options appraisal was conducted and incurable STIs/STDs such as Herpes simplex virus 2 (HSV 2), Hepatitis B and Hepatitis C were considered but fall outside the scope of this intervention. Male sex workers (MSW) were not included as a target population in this intervention as they are a distinct population with differing risk profiles and patterns of healthcare engagement.

Overall Goal:

To reduce the prevalence of curable STIs by 30% CT (16.4% to 11.5%), NG (1.6% to 1.1%), trichomoniasis (8% to 5.6%) and Syphilis (7.3% to 5.1%), amongst female sex workers over a three-year period

Objectives:

1. Increase the number of FSWs receiving STI testing every 6 months from approximately 35% to 60% over three years
2. Ensure that at least 90% of FSWs diagnosed with an STI receive treatment within 7 days
3. To increase uptake of integrated sexual health services, including PrEP and cervical screening, and strengthen access to referral pathways for FSWs

Intervention Outline

The proposed intervention is a 3-year, community based STI screening and treatment programme targeting female sex workers (FSWs) in the Lima Metropolitan Area. The intervention responds to gaps observed in STI screening and treatment carried out by existing clinics by delivering flexible services that aim to reach approximately 5,000 unique FSWs annually, which represent approximately 70-75% of the estimated FSW population with additional repeat visits for follow up. The intervention will consist of four interlinked work packages designed to improve testing and treatment rates and provide support. It also adopts a peer-led and partnership-based approach by working closely with the Ministry of Health (MINSA) and local organisations such as Miluska Vida y Dignidad (MVD) to ensure that the services are accessible and appropriate. Service delivery shall be decentralised by use of two mobile clinics and outreach support, but there will also be an option to link to existing health facilities.

Work Package 1: Community Mapping, Engagement, Peer Outreach

The programme will begin by mapping approximately 120 sex work hotspots across Lima which will include brothels, bars, clubs, street locations and other informal venues in operation. This will identify key areas and observe patterns of mobility amongst FSWs. The programme will begin by recruiting roughly 25 female peer educators taken primarily from the local sex worker community. The peer educators will play an important role by conducting outreach in target locations. This will be done by engaging with women already connected to services and those working outside the regulated systems. We would aim for each peer educator to reach approximately 25-30 FSWs a month which would give a total outreach of 7,500-9,000 sex workers a year.

After six months the programme will scale up by recruiting a further 5 peer educators to bring total contacts reached to 9,000-10,500 a year. In this situation repeat contacts will be expected and encouraged in order to increase follow up. Peer educators will focus on promoting STI testing (every 6 months, with more frequent quarterly testing encouraged for individuals at higher risk, including those with recent STI diagnoses or initiating PrEP), raise awareness of PrEP and cervical screening and support linkage to other services such as Via Libre which focuses primarily on HIV services [15]. Although MINSA recommends monthly and quarterly testing this may not be feasible for informal FSWs and therefore we highly encourage quarterly testing where and if possible [6].

Work Package 2: Integrated STI, HIV and HPV services and Connection with Stakeholders

Two mobile clinics will be equipped to deliver integrated sexual health services. This shall be carried out in partnership with MINSA and MVD who will we conduct stakeholder meetings with. These services will operate flexible hours in order to facilitate the needs of the FSWs. The clinic will be open 5 days a week and aim to reach 5,000 unique FSWs annually, with additional repeat visits for follow up and ongoing care. The clinics will offer screening for syphilis, HIV chlamydia, trichomoniasis and gonorrhoea. In addition to STIs, the programme will diagnose and manage common non-sexually transmitted infections such as bacterial vaginosis and candidiasis, which are highly prevalent amongst FSWs and can increase overall susceptibility to STIs [11]. Cervical screening will be provided through Pap smears and any detection of abnormalities will lead to a referral to advanced services. The results of the screening will be delivered on the same day where possible to reduce loss to follow up and same day treatment will also be offered to ensure that over 90% of FSWs diagnosed with an STI receive treatment within 7 days. The services will be free of charge, confidential and will not require any formal registration in order to protect the privacy of those accessing the service. In the event someone tests positive for cervical cancer or HIV, referral pathways will be established with the MINSA primary health care centres and ART clinics for more advanced care.

Work Package 3: HCW training and strengthening pre-existing systems

Healthcare workers (HCWs), including doctors, nurses and clinical support staff will be sourced from the local community and trained in stigma-free and FSW sensitive care. A cascade training programme will be put in place, where a designated HCW will train other HCWs after receiving a comprehensive training programme conducted in the two designated clinics. One clinician will be trained initially in each of the clinics with scale up to 2 clinicians per clinic after 6 months. Doctors will be trained to lead clinical decision making, STI diagnosis and referral coordination and nurses will conduct screening and administer treatment. We will also provide a training outline to the collaborating clinics in order to integrate the training into routine practice that extends well past the end of the programme. Standardised clinical protocol will be enforced to ensure good service delivery across the two clinics.

Work Package 4: Prevention and Information Education and Communication (IEC)

This package will conduct a thorough prevention and IEC programme to increase uptake of services. This component will complement the peer outreach mentioned in WP1 and aid in service delivery by addressing previously mentioned barriers to care. Peer educators will deliver targeted health education focusing on regular testing, risk reduction counselling, condom promotion and the importance of adhering to treatment [14]. This will also aid in reducing possible AMR from improper antibiotic use. IEC materials such as leaflets will be created and a social media awareness campaign will be run on applications such as Instagram, Facebook and TikTok to support these messages and spread sexual health information. Prevention activities will include distribution of approximately 200,000 condoms annually, alongside lubricants in order to promote STI prevention and peer educators will also offer guidance on condom negotiation. A stigma

reduction campaign will also be implemented in collaboration with MINSA and MVD. This campaign will engage with local healthcare providers to promote inclusive, FSW-sensitive care.

Monitoring and Evaluation of the Intervention

Good monitoring and evaluation of the efficiency and reach of the intervention is essential for the programme's success. The M&E frame will combine routine monitoring with periodic evaluations conducted every quarter in order to assess inputs, outcomes and the scale of the impact the programme is having. There is a dedicated M&E team consisting of three staff who will conduct collection of data, analyse these results and ensure that there is consistency across the two mobile clinics. Monitoring will be carried out analysing data collected from the clinics, peer outreach activities and partner health facilities (ART clinics and women's health centres). The programme is designed that indicators correspond to programme objectives and include (1) the number of FSWs reached through peer-led outreach; (2) the number of FSWs receiving STI testing, (3) the proportion of STI positive FSWs that receive treatment within 7 days of diagnosis and (4) the uptake of integrated services. The data will be collected using standardised tools such as outreach logs, clinic registers, surveys and referral tracking forms. Anonymized identifiers will be utilised to track repeat visits to the clinics whilst respecting the privacy of the patients. Data will be entered onto an electronic database, cleaned and validated to minimise error. Monitoring will be conducted monthly and programme reviews will be held quarterly, these reviews will look at the programmes progress and compare against the key targets as mentioned previously.

To gage effectiveness, a baseline, midline (18 months) and endline (36 months) evaluation will be conducted. These evaluations will incorporate qualitative and quantitative methods. Bio-behavioural surveys will be conducted after visiting the clinics to measure treatment outcomes, testing coverage, behavioural changes and any uptake of integrated services on offer. Biological sampling will be incorporated at baseline and endline to measure the prevalence of curable STIs. This allows for assessment of progress towards the programme goal of achieving a 30% reduction in curable STI prevalence over 3 years. The primary outcome indicator will be the proportion of FSWs reporting STI testing within the last 6 months. Where possible, additional analysis will assess more frequent testing (every three months) amongst FSWs operating in the formal sector in line with national guidelines. Secondary outcome indicators will include treatment coverage within 7 days, PrEP uptake, cervical screening and referral completion rates. Midline findings (18 months) will be used to alter certain parts of the programme if needed e.g., hiring and training of more staff or modification of service delivery models. To ensure high quality data, numerous quality assurance checks will be conducted including routine supervision visits and data auditing. Standard operating procedures (SOPs) will be implemented for data entry and storage to ensure the data is uniformly entered and easy to read. The data will be safely stored in a password protected systems with no access granted outside the programme. The data collected from the programme will be used to identify possible gaps in the service delivery such as possible low uptake in certain regions in Lima. By observing gaps, it allows for adjustments to the programme to reflect the findings.

Findings from the programme will be shared with stakeholders, including the Ministry of Health (MINSA) and MVD in order to support coordination. Due to the sensitive nature of the programme all M&E activities will adhere to strict ethical standards. Ensuring the confidentiality and anonymity of the FSWs is paramount and given the stigma and potential legal and social risks associated with sex work, special care will be taken to ensure that the FSWs do not feel pressured to participate in the programme. No formal registration will be required to access the services and informed consent will be collected for all survey-based data acquired.

Budget & Justification of use of resources

This proposal seeks £10 million GBP in funding which is equivalent to approximately 46,225,694.33 PEN[15.] This adequately reflects the funding needed to deliver a comprehensive STI screening, treatment and prevention programme targeting FSWs over a 3-year period. The largest proportion of funding will be allocated to consumables (23%) and service delivery (22.5%) this includes purchase of STI diagnostics (syphilis rapid tests, gonorrhoea /chlamydia tests), medication, antibiotics, vaginal swabs, metal speculums, condoms, lubricants, training manuals and mobile clinic stocks. This investment is essential to achieve the desired high coverage across the city and reduce the number of asymptomatic and untreated infections. 10.5% is dedicated to staffing allowing for good programme management, strong community-based workforce, clinical teams, outreach coordinators, peer educators and M&E staff. M&E was allocated 8.5% which will allow for robust data management and collection of baseline, midline and endline data. This can lead to effective goal planning, adjustment of goals if needed and strong overall data analysis. 3% was allocated towards travel to cover insurance, fuel, tax and possible repairs for the mobile clinics. Due to current conflicts an 8% inflation rate in the price of fuel was taken into account. The travel budget also covers peer-educator travel to allow them to go to difficult to access locations in order to improve outreach and biannual visits to MINSA headquarters for stakeholder meetings. Training and capacity strengthening (1.4%) will be carried out primarily in year 1 with refresher training provided at 16 and 24 months, while overheads (10%) will cover essential operations that culminate in optimised full coverage services. 10.5% has been allocated to an emergency reserve in case the programme needs to scale up on interventions and 8.5% was allocated to sustainability to ensure continued service provision after the programme has ceased. The budget allocation increases in year 3 to reflect sustainability planning.

Table 1: Programme Budget Summary

Category	**Year 1**	**Year 2**	**Year 3**	**Total**
Staffing	315,000	350,750	384,250	1,050,000
Service Delivery	750,000	740,000	760,000	2,250,000
Travel	100,000	100,000	100,000	300,000
Training	80,000	45,000	15,000	140,000
Consumables	650,000	850,000	800,000	2,300,000
Sustainability	225,000	300,000	325,000	850,000
M & E	250,000	300,000	300,000	850,000
Overheads	358,800	388,700	402,500	1,150,000
Emergency reserve	200,000	350,000	500,000	1,050,000
Total	2,928,800	3,424,450	3,586,750	10,000,000

References

1. Miller GA, Mendoza W, Krone MR. Clients of female sex workers in Lima. Peru: a bridge population for sexually transmitted disease/HIV transmission. Sex Transm Dis. 2004; 31:337–42. doi: 10.1097
2. Pruss-Ustun A, Wolf J, Driscoll T. HIV due to female sex work: regional and global estimates. PLOS One. 2013;8:e63476. doi: 10.1371/journal.pone.0063476
3. Kinsler JJ, Blas MM, Cabral A, Carcamo C, Halsey N, Brown B. Understanding STI Risk and Condom Use Patterns by Partner Type Among Female Sex Workers in Peru. Open AIDS J. 2014 May 30;8:17-20. doi: 10.2174/1874613601408010017. PMID: 24949112; PMCID: PMC4062928.
4. The path that ends AIDS: UNAIDS Global AIDS Update 2023. Geneva: Joint United Nations Programme on HIV/AIDS; 2023. Licence: CC BY-NC-SA 3.0 IGO
5. Cárcamo CP, Campos PE, García PJ, Hughes JP, Garnett GP, Holmes KK; Peru PREVEN study team. Prevalences of sexually transmitted infections in young adults and female sex workers in Peru: a national population-based survey. Lancet Infect Dis. 2012 Oct;12(10):765-73. doi: 10.1016/S1473-3099(12)70144-5. Epub 2012 Aug 8. PMID: 22878023; PMCID: PMC3459082.
6. Huamaní-Chavez V, Rueda-Torres L, Ormeño-Delgado L, Li J, Rosales-Rimache J. Syphilis seroprevalence and associated factors: A cross-sectional study in formal female sex workers in a province of Peru. Medicine (Baltimore). 2024 Apr 12;103(15):e37774. doi: 10.1097/MD.0000000000037774. PMID: 38608070; PMCID: PMC11018198.
7. Davtyan M, Munoz K, Urada L. Transactional sex: a client's perspective from Peru. Available from http://www.ejhs.org/volume16/Transaction.htmlElectronic J Hum Sex. 2013;16:1–8
8. Montano SM, Hsieh EJ, Calderon M. Human papillomavirus infection in female sex workers in Lima Peru. Sexual Transm Infect. 2011;87:81–2. doi: 10.1136/sti.2010.043315
9. Bruni L, Albero G, Serrano B, Mena M, Gómez D, Muñoz J, et al. Human papillomavirus and related diseases in the world-summary report. ICO/IARC Inf Cent HPV Cancer (HPV Inf Centre) 2019a;(June)307
10. Aguilar A, Pinto JA, Araujo J, Fajardo W, Bravo L, Pinillos L, Vallejos C. Control of cervical cancer in Peru: Current barriers and challenges for the future. Mol Clin Oncol. 2016 Aug;5(2):241-245. doi: 10.3892/mco.2016.926. Epub 2016 Jun 10. PMID: 27446557; PMCID: PMC4950606.
11. 6.Scorgie F, Chersich MF, Ntaganira I, Gerbase A, Lule F, Lo YR. Socio-Demographic Characteristics and Behavioral Risk Factors of Female Sex Workers in Sub-Saharan Africa: A Systematic Review. AIDS Behav. 2011 doi: 10.1007/s10461-011-9985
12. Kohler PK, Campos PE, Garcia PJ, Carcamo CP, Buendia C, Hughes JP, Mejia C, Garnett GP, Holmes KK. Sexually transmitted infection screening uptake and knowledge of sexually transmitted infection symptoms among female sex workers participating in a community randomised trial in Peru. Int J STD AIDS. 2016 Apr;27(5):402-10. doi: 10.1177/0956462415584488. Epub 2015 May 4. PMID: 25941053; PMCID: PMC4742423.
13. Workowski KA, Berman S. Sexually transmitted diseases treatment guidelines. MMWR Recomm Rep. 2010;59(RR-12):1–110 bv refernc
14. Campos PE, Buffardi AL, Cárcamo CP, García PJ, Buendia C, Chiappe M, Garnett GP, Xet-Mull AM, Holmes KK. Reaching the unreachable: providing STI control services to female sex workers via mobile team outreach. PLoS One. 2013 Nov 25;8(11):e81041. doi: 10.1371/journal.pone.0081041. PMID: 24282565; PMCID: PMC3839873.
15. https://vialibre.org.pe/

Appendices

Project summary	Verifiable indicators	Sources of verification	Frequency	Important assumptions
Goal To reduce the prevalence of curable STIs by 30% CT (16.4% to 11.5%), NG (1.6% to 1.1%), trichomoniasis (8% to 5.6%) and Syphilis (7.3% to 5.1%), amongst female sex workers over a three-year period	Prevalence of curable STIs (chlamydia, gonorrhoea ,trichomoniasis and syphilis) amongst FSW	Collecting Laboratory data throughout the duration of the programme Bio- behavioural surveys	Baseline and endline (36 months)	FSWs remain available throughout the duration of the project and don't move away, no major policy disruptions, no major service disruptions
Outcome/objective Increase access and uptake of STI screening ,testing and treatment amongst FSWs	-Percentage of FSWs who received STI testing in the last 6 months (increase from 35% to 60%) -Percentage of FSW diagnosed with STIs receiving treatment within 7 days (target is ≥90%) (indicators will be separated by formal/informal sector where possible)	Clinic registers, mobile clinic records and behavioural surveys	Annually (routine monitoring quarterly)	Services are available, free of charge, confidential and accessible to FSWs
Outputs Community-based STI services for FSWs delivered by mobile clinics	-Number of FSWs reached with STI services (STI services are regarded as a screening, treatment and prevention package) -Number of FSWs receiving STI testing annually (˜roughly 9000 a year)	Mobile clinic records, outreach logs and service delivery reports	Quarterly	Outreach teams are able to access hotspots and the demand for the services are sustained, partnerships with NGOs, FSW organisations and stakeholders remain functional
Inputs / Activities Hotspot mapping, peer outreach, mobile clinic set-up, HCW training, condom and lubricant distribution, PrEP distribution and cervical screening	Percentage of planned outreach sessions and mobile clinics implemented as planned (greater than 90%)	Activity reports, training schedule and supervision logs	Monthly	Staffing and supplies remain available and supply chains are able to function without disruption

Figure 1: LogFrame

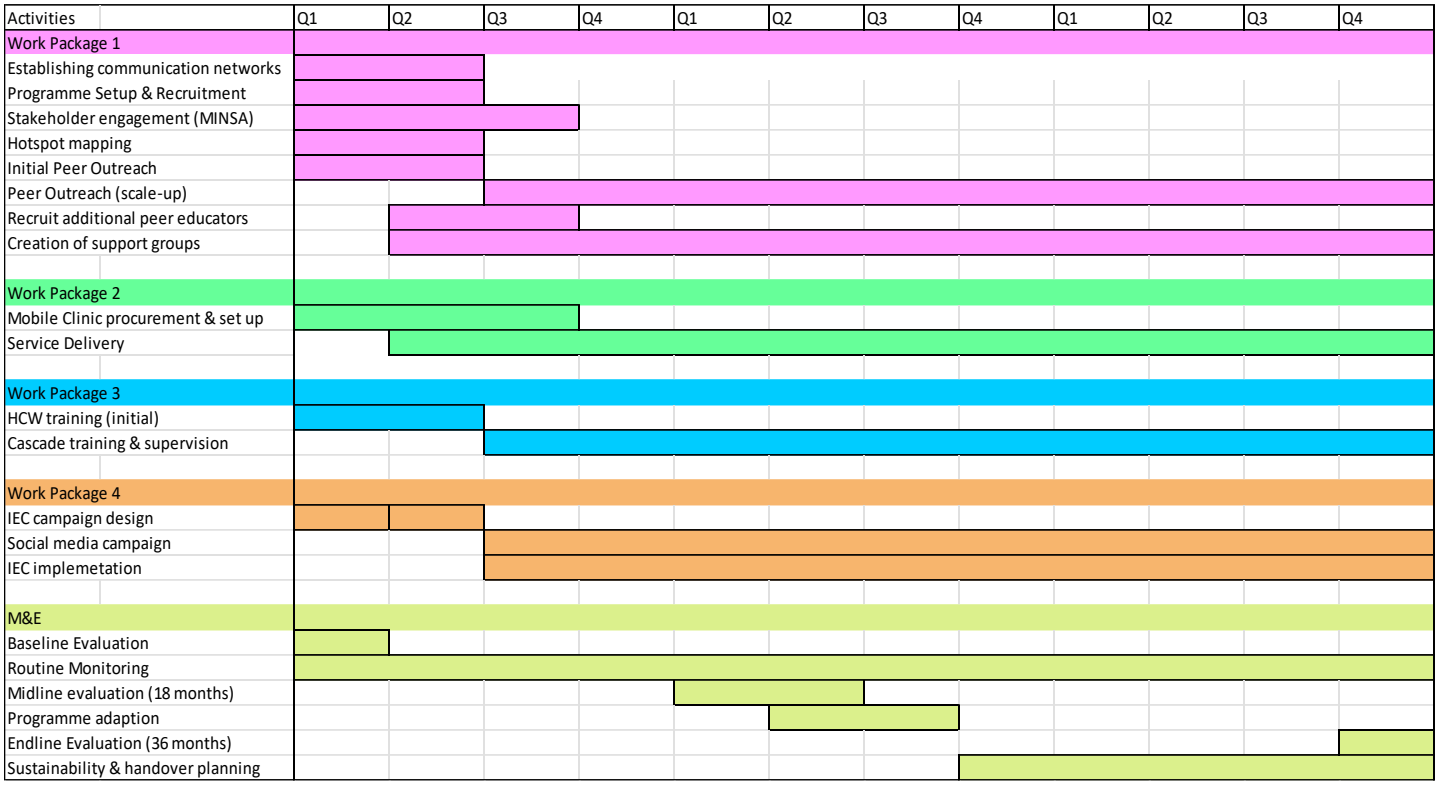


Figure 2: Gantt Chart